# PATIENT CHART EXTRACT - VICTORIA GENERAL HOSPITAL (TEST)

**MADE-UP TEST PATIENT - NOT A REAL PERSON - FOR SANDBOX TESTING ONLY**

Patient: TESTPATIENT, Harold  DOB: 1952-02-11  Sex: M
MRN: TEST60922  Location: 5B Orthopedics, Bed 4
Admitted: 2026-11-06 (Orthopedics)  Attending: Dr. B. Bones (made-up)

## ORTHOPEDICS ADMISSION NOTE

Date of service: 2026-11-06 14:10
Chief complaint: Left hip pain after a fall.

### HISTORY OF PRESENT ILLNESS

74-year-old man who slipped while stepping off a curb at walking pace and landed on his left side. Unable to weight-bear. No head strike, no loss of consciousness. Family reports 3 months of mid-back pain, fatigue and about 6 kg weight loss. Seen by family doctor for the back pain, treated as mechanical. Family doctor letter: Creatinine 92 umol/L on Oct 2, 2025.

### PAST MEDICAL HISTORY

Hypertension
Type 2 diabetes (diet controlled)
Osteoarthritis of both knees
No known cancer.

### HOME MEDICATIONS

Ramipril 10 mg PO daily
Naproxen 500 mg PO bid prn back pain
Acetaminophen 1000 mg PO tid

ALLERGIES: No known drug allergies

### EXAMINATION

2026-11-06 13:40  BP: 152/88  HR: 92  RR: 18  Temp: 36.9 C  SpO2: 95%  Weight: 78 kg
Left leg shortened and externally rotated. Distal neurovascular status intact. Tender over the mid thoracic spine.

### ASSESSMENT AND PLAN

Left subtrochanteric femur fracture after a low-energy fall.
AKI likely from dehydration after the fall and naproxen. IV fluids.
Anemia likely from fracture-related blood loss.
Plan intramedullary nail tomorrow. Hold ramipril and naproxen.
Medicine/hospitalist consult for pre-operative medical optimization.

## LABORATORY RESULTS

Collected: Nov 6, 2026 12:50

| Test | Result | Units | Reference | Flag |
|---|---|---|---|---|
| Hemoglobin | 98 | g/L | 135 - 175 | L |
| MCV | 92 | fL | 80 - 100 | |
| White Blood Cell Count | 6.1 | x10^9/L | 4.0 - 11.0 | |
| Platelets | 168 | x10^9/L | 150 - 400 | |
| Sodium | 136 | mmol/L | 135 - 145 | |
| Potassium | 4.8 | mmol/L | 3.5 - 5.0 | |
| Creatinine | 168 | umol/L | 60 - 110 | H |
| Calcium | 2.92 | mmol/L | 2.10 - 2.55 | H |
| Albumin | 30 | g/L | 35 - 50 | L |
| Total Protein | 92 | g/L | 60 - 80 | H |
| Glucose | 7.4 | mmol/L | 3.6 - 7.8 | |
| INR | 1.0 | ratio | 0.8 - 1.2 | |

## ORTHOPEDICS PROGRESS NOTE - HD2

Date of service: 2026-11-07 07:50
Pain controlled. For OR today, booked as second case.
2026-11-07 07:30  BP: 138/80  HR: 88  RR: 18  Temp: 37.0 C  SpO2: 95%
Creatinine improving slowly with fluids. Medicine consult pending.
X-rays: fracture only, otherwise unremarkable.
Consent signed for right femur intramedullary nail.

## LABORATORY RESULTS

Collected: Nov 7, 2026 05:30

| Test | Result | Units | Reference | Flag |
|---|---|---|---|---|
| Hemoglobin | 94 | g/L | 135 - 175 | L |
| Sodium | 137 | mmol/L | 135 - 145 | |
| Potassium | 4.6 | mmol/L | 3.5 - 5.0 | |
| Creatinine | 151 | umol/L | 60 - 110 | H |
| Calcium | 2.88 | mmol/L | 2.10 - 2.55 | H |
| Magnesium | 0.84 | mmol/L | 0.70 - 1.00 | |
| Phosphate | 1.21 | mmol/L | 0.80 - 1.45 | |

## ACTIVE MEDICATION ORDERS (2026-11-07)

Acetaminophen 1000 mg PO q6h
Hydromorphone 0.5 mg PO q4h prn pain
Enoxaparin 40 mg SC daily (held from midnight for OR)
Normal saline 100 mL/h IV
Ramipril 10 mg PO daily (on hold)

## ACTIVE PROBLEM LIST

Hip fracture, left subtrochanteric
Acute kidney injury
Hypertension
Type 2 diabetes

## CONSULT REQUEST - MEDICINE / HOSPITALIST

Date of service: 2026-11-07 08:10
From: Orthopedics. Reason for consult: pre-operative medical optimization for hip fracture fixation today. AKI improving. Please clear for OR.

## HOSPITALIST CONSULT NOTE (DRAFT, INCOMPLETE)

Date of service: 2026-11-07 09:30
Consulted by Orthopedics for pre-operative assessment.
2026-11-07 09:15  BP: 140/82  HR: 90  RR: 18  Temp: 37.1 C  SpO2: 95%
Low-energy fall with a subtrochanteric (not neck of femur) fracture.
Back pain and weight loss for 3 months. High calcium, new kidney injury, anemia on arrival before much blood loss, and a wide gap between total protein and albumin.
Radiology called the fracture-site lucency osteopenia and the rib lesion an old fracture.
Possible pathological fracture? Myeloma?
To discuss with Orthopedics before fixation.

## RADIOLOGY REPORTS

### X-RAY LEFT HIP AND FEMUR

Date of exam: 2026-11-06 12:20

**FINDINGS:**

Displaced subtrochanteric fracture of the left femur.
Ill-defined lytic lesion at the fracture site, favoured to represent osteopenia.
Generalized osteopenia. Mild degenerative change of the left hip joint.

**IMPRESSION:**

1. Displaced left subtrochanteric fracture.
2. Lucency at the fracture site likely osteopenic bone. Clinical correlation.

### X-RAY CHEST (PRE-OPERATIVE)

Date of exam: 2026-11-06 12:35

**FINDINGS:**

Lungs clear. Small left pleural effusion. Normal heart size.
Expansile lytic lesion of the right 6th rib, likely an old healed fracture.
Mild compression deformity of T8, age indeterminate.

**IMPRESSION:**

No acute cardiopulmonary process. Possible old rib fracture.
ADDENDUM (2026-11-07 15:40): Compared with the chest X-ray of 2025-10-02, the expansile lesion of the right 6th rib is new. Recommend further evaluation for a lytic lesion (myeloma or metastasis).

### X-RAY CHEST (FAMILY DOCTOR REFERRAL)

Date of exam: 2025-10-02 11:00

**FINDINGS:**

Lungs clear. Normal heart size. Ribs intact. No focal bone lesion.
Thoracic vertebral body heights are preserved.

**IMPRESSION:**

Normal chest.